LIBBY

Question report

q-aml-x7q2-cr-induction-svl7

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Question report

> Which remission-directed therapy, deliverable now as her next line, has the best evidence for inducing a complete remission (ELN 2022 CR; CRi, CRh, MLFS and composite rates reportable only when labelled as what they are) in relapsed/refractory AML-MR matched to her situation: relapse six years after a matched-sibling allograft, refractory to FLAG-IDA plus venetoclax, roughly 85% marrow blasts, complex adverse karyotype, and a right-thigh myeloid sarcoma whose response counts toward the remission?

No moderate confidence

This question is about the published case aml-mds-related-rr-tp53-aberrant-hla-pending-x7q2-rerun2. That case's ranking was not revisited and stands as published.

No, with moderate confidence: no therapy deliverable to her now has the best evidence for inducing a complete remission, for two reasons at once. On demonstrated true CR in every population containing her, the confidence intervals all overlap; nothing separates. And ELN 2022 writes absence of extramedullary disease into the CR definition itself, so her documented thigh myeloid sarcoma fails every marrow-only figure against the question's own endpoint. The table below holds all 24 therapies the research tier assessed, in two segments, neither ordered by CR probability: deliverable rows by eligibility, deliverability and population match; the ruled-out tail by non-viability. The tail is the point, not padding. An absent row reads as never-considered, so the failures appear with their reasons showing: a phase 3 terminated for missing its endpoint, a response definition that was never ELN CR, a randomised trial that excluded prior transplant, doors closed to her at entry. Why a candidate failed is often more useful than why the survivors passed, and none of it is a prescription: the table shows the numbers behind the "no" without choosing among them.

Two rows now carry the thigh lesion directly, weighted above their evidence tier for exactly that reason. The intensive-chemotherapy row holds the largest adult myeloid sarcoma series with a remission figure, 45% CR, but its source does not say whether extramedullary sites were separately assessed at response, so it cannot be read as a marrow-plus-tissue CR. The venetoclax-HMA row is pooled, never resolved to true CR, and her own venetoclax refractoriness argues against re-exposure.

Read the verdict precisely. It is not "we don't know": the matched populations are studied, and they answer negatively. Nor "nothing can be done": every deliverable candidate keeps a real, undifferentiated chance of remission, and three of five board members ranked CLAG-GO on NCT04050280 first, the one open protocol written to admit her, an eligibility pick rather than a CR rate it has never published. The best-evidenced candidate still has no route: lintuzumab-Ac225 plus CLAG-M is stranded solely by deliverability, and the expanded-access call that could change that has never been made; one ranking changes on a yes. The one on-label option, fractionated gemtuzumab, is placed by access, not efficacy.

No guideline requires a CR before a second transplant, so going straight to sequential conditioning is a live alternative to buying a remission first; the board questioned the goal, not just the options. When demonstrated benefit stops discriminating, cost does: the one randomised test of induction against none, which excluded prior allografts, priced it at 61% versus 21% severe toxicity and 27 extra hospital days for 3.4 points of day-56 CR; its figures live in the sequential-conditioning row, ASAP being a strategy rather than a therapy. Nobody has asked her the two questions that decide this: whether a 13 to 17% chance of the intended result is worth 7 to 21% sixty-day mortality, and whether she would accept conditioning without a CR first. Whatever is chosen, thigh imaging belongs in response assessment or no CR claim about her can be checked.

This run assessed one endpoint, CR induction. Durability, survival and everything after remission belong to the source case, whose ranked dossier was not revisited and stands as published.

No. No therapy deliverable to her now has the best evidence for inducing an ELN 2022 complete remission, because the published evidence does not discriminate among the deliverable candidates and none of it measures the endpoint as this question defines it. On demonstrated true CR in populations that contain her, every confidence interval overlaps every other: CPX-351 plus venetoclax 15% (5/33, Wilson 6.7 to 30.9), venetoclax-based salvage after allograft 26.3% (5/19, 11.8 to 48.8), the randomised MIRROS control arm 17.1% (21/123, 11.4 to 24.7), and the 594-patient second-salvage benchmark 13% (10.3 to 15.7). The figures that do clear that pack are either composites with no CR split (HAM plus venetoclax, CRc 75%) or belong to populations she is not in (CLAG-M 58%, pre-venetoclax with no post-allograft patients; FLAMSA 88%, mostly first transplants). And because ELN 2022 writes absence of extramedullary disease into the CR definition itself, every marrow-only figure in this dossier fails the question's own definition for a woman with a documented thigh myeloid sarcoma; the only tissue-response evidence found is two paediatric patients imaged on gemtuzumab, with no CR grading. What survives this negative verdict is a convergence, not a winner. The guideline position (NCCN routes relapsed/refractory disease to a clinical trial, and no guideline names a regimen for second salvage after intensive chemotherapy plus venetoclax in a post-allograft patient) and the evidence position arrive at the same place: a trial slot. Three of five board personas put CLAG-GO on NCT04050280 first, on eligibility and deliverability rather than on a CR rate it does not have; it is the one open protocol written to admit her (post-allograft relapse from day 60, no blast or WBC cap), and its entry screen supplies the organ-function documentation this case is missing. Two findings must travel with that. First, once demonstrated benefit stops discriminating, what separates the options is cost: ASAP priced the buy-a-remission-first route at 61% versus 21% grade 3 or worse non-haematological adverse events and a median 27 extra hospital days for a day-56 CR difference of 3.4%. Second, no guideline writes a CR precondition into a second transplant, so proceeding to sequential conditioning (NCT06928662, NCT04375631) without buying a remission first is a live alternative that should be put to her rather than assumed away. Nobody has yet offered her the real trade, a 13 to 17% chance of the intended result against 7 to 21% sixty-day mortality, and asked whether she wants it; whatever is chosen, the thigh lesion needs imaging at response or no resulting remission can be graded against the definition she asked about.

Candidates assessed

!!! note "What this ranking orders by" Eligibility, deliverability and population match of the evidence, in that order, following the board's practical convergence. This order is explicitly not demonstrated CR probability: on true CR in every population that contains her, the candidates' confidence intervals all overlap (acceptance criterion 4), so no ordering by the endpoint the question asks is supportable, and the verdict above is 'no' for exactly that reason. Rows 1 and 2 are the two protocol routes open to her; rows 3 to 12 are deliverable off-protocol options ordered by how directly their evidence reaches her, with the two myeloid-sarcoma rows (11 and 12) carried above their evidence tier because the thigh lesion is why every marrow-only figure fails her endpoint; row 13 is the randomised benchmark kept for context and carries the second-salvage floor in its notes. Rows 14 to 24 were assessed and are not answers: the tail is ordered by non-viability, first the stranded agents with real signals and no route (14 to 19, strongest matched signal first), then registry-only protocols whose doors are closed to her or unopened (20 to 23), and last a randomised platform that adds no distinct therapy (24). An absent row would read as never-considered, which is why the ruled-out candidates appear at all. The table shows the numbers that produced the negative verdict; it does not prescribe among them.

<table class="trial-table"><thead><tr><th>Rank</th><th>Candidate</th><th>Response rate</th><th>Toxicity</th><th>Available to her</th><th>Key references</th></tr></thead><tbody><tr><td>1</td><td>CLAG-GO (cladribine, high-dose cytarabine, G-CSF, fractionated gemtuzumab ozogamicin) on NCT04050280
<small class="persona-line">The board's practical convergence: three of five personas ranked it first, on eligibility and deliverability rather than on a CR rate it does not have. Its entry screen supplies the organ-function documentation this case is missing. Two gates remain unverified in her chart: CD33 on at least 20% of blasts, and the creatinine and bilirubin thresholds.</small></td><td>No published rate in refractory AML; backbone proxy CLAG-M 58%
<small class="persona-line">CR (true CR, borrowed from the CLAG-M backbone; CLAG-GO itself has zero published results in this indication)</small>
<small>66/114 evaluable (CLAG-M proxy; the often-printed 66/118 gives 55.9%)</small>
<small>95% CI 95% CI 48.7 to 66.6 (Wilson, computed, on the borrowed backbone figure)</small>
<small class="persona-line">The proxy figure comes from the 2008 Polish CLAG-M cohort: pre-venetoclax, no post-allograft subgroup, CR scored by pre-ELN morphologic criteria, and WBC above 10 g/L predicted failure where her peripheral blasts ran 58 to 82%. The trial protocol, by contrast, was written to admit her: post-allograft relapse eligible from day 60, no blast or WBC cap. Its only efficacy publication is frontline core-binding-factor disease, the wrong population entirely.</small></td><td>Unpublished for this combination;

CLAG-M backbone: severe infection 45%, early death 7%, grade 4 cytopenias lasting about three weeks in responders; gemtuzumab adds a boxed hepatotoxicity/VOD warning.
<small class="persona-line">All figures borrowed from the CLAG-M backbone cohort and the gemtuzumab label; the trial has published no safety data of its own.</small></td><td>Trial only</td><td>NCT04050280
PMID 18076637
PMID 21109304</td></tr><tr><td>2</td><td>Sequential conditioning into a second allograft (NCT04375631 or NCT06928662)
<small class="persona-line">The live alternative no guideline forecloses: nothing writes a CR precondition into a second transplant, so proceeding to conditioning without buying a remission first should be put to her rather than assumed away. NCT04375631 admits refractoriness defined by failed venetoclax combinations and offers a cladribine debulk instead of re-exposing her to the failed FLAG-Ida. ASAP is this row's strategy evidence rather than a separate candidate: its randomisation tested exactly this route's premise, that the conditioning can produce the remission.</small></td><td>88% (FLAMSA analogue)
<small class="persona-line">CR (true CR, produced by the whole procedure of sequential chemotherapy, conditioning and graft, not by reinduction alone; not comparable with standalone reinduction rates)</small>
<small>66/75 (FLAMSA, 2005)</small>
<small class="persona-line">Overwhelmingly first transplants two decades before venetoclax-era salvage. ASAP, the randomised support for skipping induction (day-56 post-transplant CR 83% without induction versus 79% with), excluded prior allografts and did not show non-inferiority (p=0.036). Neither open platform carries a remission endpoint, so her scenario will never get a CR figure from these protocols.</small></td><td>High but unquantified for her scenario: second-graft non-relapse mortality in active-disease post-allograft relapse is unpublished on either platform. ASAP's cost ledger favours the route: grade 3+ non-haematological AEs 21% with watchful waiting versus 61% with induction, and a median 27 fewer hospital days.
<small class="persona-line">ASAP figures come from patients heading to a first allograft; prior allo-HCT was excluded.</small></td><td>Trial only</td><td>NCT04375631
NCT06928662
PMID 16110027
PMID 38583455</td></tr><tr><td>3</td><td>Hypomethy agent plus venetoclax, post-allograft relapse (off protocol)
<small class="persona-line">The matched true-CR anchor for this strategy is the Vanderbilt post-allograft series: 26.3% (5/19 evaluable, Wilson 11.8 to 48.8), one of the few post-transplant venetoclax series to separate CR from CRi. Venetoclax's own label covers newly diagnosed unfit AML only.</small></td><td>48% (40/83)
<small class="persona-line">CRc (composite CR/CRi; no CR-only split published)</small>
<small>40/83</small>
<small class="persona-line">Closest published population on the transplant axis, and the only series where prior venetoclax failure was tested against outcome and did not predict it. But median relapse came at 7 months post-transplant with low disease burden, against her six years and roughly 85% marrow blasts, and response was assessed on marrow alone.</small></td><td>Lowest early mortality in the table: 30-day mortality 3.6% (all infectious), against near-universal grade 3-4 cytopenias (99%) and febrile neutropenia 24%.</td><td>Off-label</td><td>PMID 42499388
PMID 32390196</td></tr><tr><td>4</td><td>CLAG-M off protocol
<small class="persona-line">The file's only head-to-head leans its way: CLAG 37.9% versus MEC 23.8% true CR (p=0.048), from a single-centre unadjusted retrospective with physician-assigned treatment, too weak to rank on.</small></td><td>58%
<small class="persona-line">CR (true CR by the era's morphologic criteria, pre-ELN, after one or two courses)</small>
<small>66/114 evaluable (the often-printed 66/118 gives 55.9%)</small>
<small>95% CI 95% CI 48.7 to 66.6 (Wilson, computed)</small>
<small class="persona-line">The largest true-CR figure on a deliverable backbone, from a population she is not in: the 2008 cohort closed before venetoclax existed, reports no post-allograft subgroup, and WBC above 10 g/L predicted failure where her peripheral blasts ran 58 to 82%.</small></td><td>High: severe infection 45%, early death 7%, and grade 4 cytopenias lasting a median of just over three weeks in responders.</td><td>Off-label</td><td>PMID 18076637
PMID 21109304</td></tr><tr><td>5</td><td>CPX-351 plus venetoclax on NCT03629171
<small class="persona-line">Vyxeos is labelled for newly diagnosed t-AML/AML-MRC only, so off-protocol use in second salvage is off-indication; the option survives only as a slot on the still-recruiting protocol. One board persona ranked it first as the only prospective cohort that enrolled her kind of patient and still published a true CR; the same persona's interval analysis undoes that ranking along with every other.</small></td><td>15% (5/33)
<small class="persona-line">CR (true CR; the composite CRc was 39%, and her stratum, second salvage or later or TP53-mutated, reached 26% CRc)</small>
<small>5/33</small>
<small>95% CI 95% CI 6.7 to 30.9 (Wilson, computed)</small>
<small class="persona-line">The most transferable figure in the file: the cohort had 58% prior venetoclax, 30% prior transplant and 44% at second salvage or later, which is her kind of patient. That is exactly why its 15% is the number to believe, and it is one of the lowest here.</small></td><td>The worst documented cost in the table: 60-day mortality 21%, 30-day 9%, grade 3+ febrile neutropenia 76%, bacteraemia 64%, six grade 5 events in 33 patients.</td><td>Trial only</td><td>PMID 40401707
NCT03629171</td></tr><tr><td>6</td><td>Venetoclax plus high-dose cytarabine and mitoxantrone (RELAX; HAM-VEN), off protocol
<small class="persona-line">The one deliverable figure that clears the matched pack, and it is a composite. A CR-only split that held up in the post-transplant subgroup would separate a candidate and move the verdict; the registry posts no results and the trial's recruitment status reads unknown.</small></td><td>75% (41/55)
<small class="persona-line">CRc (composite CR/CRi by intention to treat; no CR-only split published)</small>
<small>41/55</small>
<small>95% CI 95% CI 61 to 85 (as published)</small>
<small class="persona-line">The largest post-transplant fraction of any prospective salvage cohort here (19/55 with a prior allograft), but enrolment required relapse from CR or failure of standard induction; her second-salvage venetoclax-refractory state fell outside it.</small></td><td>Moderate to high: febrile neutropenia 53% (all grades), pneumonia 22%, sepsis 22%, four potentially treatment-related deaths (7.3%). The serious-adverse-event line does not reconcile: 15 SAEs in 14 patients printed as 13%, where 14/55 is 25%.</td><td>Off-label</td><td>PMID 41791831
NCT04330820</td></tr><tr><td>7</td><td>Fractionated gemtuzumab ozogamicin monotherapy (MyloFrance-1 schedule)
<small class="persona-line">On-label for relapsed/refractory CD33-positive AML. The only tissue-response evidence in the whole file rides on this agent: two paediatric patients with CD33-positive extramedullary disease shrinking on serial FDG-PET/CT, neither response graded as CR.</small></td><td>26% (15/57)
<small class="persona-line">CR (true CR; CRp 7% reported separately)</small>
<small>15/57</small>
<small class="persona-line">First relapse, transplant-naive, venetoclax-naive, single-arm: 26% is a ceiling rather than an estimate for her. The only on-label option in this table, and the board declined to carry it because on-label supply against 85% marrow blasts is the on-label way to undertreat her.</small></td><td>Comparatively low in the source series: grade 3+ neutropenia for a median 23 days, no grade 3-4 liver toxicity and no VOD in 57 patients including 7 transplanted afterwards. The label still carries a boxed hepatotoxicity/VOD warning, which matters ahead of a possible second graft.</td><td>Available</td><td>PMID 17051246
PMID 29734213</td></tr><tr><td>8</td><td>MEC (mitoxantrone, etoposide, cytarabine) off protocol
<small class="persona-line">Loses the file's only head-to-head to the cladribine backbone on true CR (23.8% versus 37.9%, p=0.048).</small></td><td>33.3% (20/60)
<small class="persona-line">CR (true CR; CRi 18.3% and composite CR/CRi 51.7% reported separately)</small>
<small>20/60</small>
<small class="persona-line">Single-centre R/R cohort in which response was strongly blast-burden dependent: marrow blasts at or below 20% and peripheral blasts at or below 30% more than doubled the odds of CR/CRi, and she sits at roughly 85% and 82%, the wrong side of both cut-points.</small></td><td>High: 35% of the cohort needed intensive care during the admission; febrile neutropenia and severe mucositis described as common with no published rates.</td><td>Off-label</td><td>PMID 38460433
PMID 21109304</td></tr><tr><td>9</td><td>CLAG (cladribine, cytarabine, G-CSF) off protocol
<small class="persona-line">The winning arm of the file's

only head-to-head (MEC 23.8% true CR, p=0.048; primary refractory subgroup 45.5% versus 22.2%, p=0.09). One
retrospective is the whole case for it; the CLAG-M row above carries the larger prospective cohort for the same backbone
plus mitoxantrone.</small></td><td>37.9%
<small class="persona-line">CR (true CR,
retrospective head-to-head against MEC)</small>
<small>97 CLAG patients of 162 in the
comparison</small>
<small class="persona-line">Single-centre retrospective with physician-assigned
treatment, split by primary refractory versus first relapse; neither prior allograft nor prior venetoclax is analysed, and her
second-salvage state fits neither stratum cleanly.</small></td><td>Unpublished: the four-page report carries no
adverse-event table and no early-mortality figure. Expect the profile of a cladribine and high-dose cytarabine backbone,
in the region of the CLAG-M rates above.</td><td>Off-label</td><td><a
href="https://pubmed.ncbi.nlm.nih.gov/21109304">PMID 21109304</td></tr><tr><td>10</td><td>Physician&
salvage after venetoclax failure, including venetoclax plus actinomycin D
<small class="persona-line">The
pessimistic anchor for the axis she is actually on: after venetoclax-based salvage fails, response of any kind ran 23% and
that figure pools CR with MLFS. The actinomycin D signal sits on six patients. Two further case reports of
venetoclax-HMA with dose-adjusted HAG were read and set aside for having no
denominator.</small></td><td>23% (5/22)
<small class="persona-line">ORR pooling CR with
MLFS; no CR-only rate is published for the whole salvaged group. The venetoclax plus actinomycin D subset reached
true CR in 3 of 6.</small>
<small>22 treated of 28; six were too unwell for any salvage attempt</small>
<small
class="persona-line">The closest published population to her exact axis: venetoclax-based treatment had failed in
all 28, 57% had a prior allograft, and the median was three prior lines. It is also sicker than she is, with 71% at ECOG 2-4
against her ECOG 1.</small></td><td>No adverse-event table is published; the tolerability signal is that six of 28
patients were too unwell for any salvage attempt. Median survival was 3.9 months overall.</td><td>Off-label</td><td><a
href="https://pubmed.ncbi.nlm.nih.gov/32997830">PMID 32997830
<a
href="https://pubmed.ncbi.nlm.nih.gov/33217852">PMID 33217852</td></tr><tr><td>11</td><td>Myeloid-sa
intensive chemotherapy (FLAI, HDAC-IDA, HyperCVAD or MEC)
<small class="persona-line">Here
because the thigh lesion is the reason every marrow-only figure in this table fails her endpoint, and this is the largest
adult myeloid sarcoma series with a remission figure attached; even it cannot say whether the sarcoma responded. The
larger comparison of transplant against chemotherapy in myeloid sarcoma (162 patients, 12 centres) publishes survival
only, so it cannot carry the extramedullary CR axis either.</small></td><td>45%
<small
class="persona-line">CR across intensively treated patients; the paper does not state whether extramedullary sites were
separately assessed at response, so the figure cannot be read as marrow-plus-tissue CR</small>
<small>43
intensively treated of 48</small>
<small class="persona-line">Nine-centre Italian registry, median age 46,
mixed presentations; the 15 secondary-AML-related cases, her category, had the worst survival of the three groups. Not
selected for post-allograft relapse or venetoclax failure.</small></td><td>Death during induction 5% among the 43
intensively treated; no other adverse-event data are published.</td><td><span class="fit-badge
fit-partial">Off-label</td><td><a
href="https://pubmed.ncbi.nlm.nih.gov/28056398">PMID 28056398
<a
href="https://pubmed.ncbi.nlm.nih.gov/39622998">PMID 39622998</td></tr><tr><td>12</td><td>Venetocla
plus hypomethylating agent directed at myeloid sarcoma
<small class="persona-line">The extramedullary
face of the rank-3 strategy, and the same review supplies the two numbers behind this table's endpoint discipline:
56% of paired sarcoma and marrow samples carry discordant mutation profiles, and 46% of post-transplant relapses in
the pooled literature were isolated extramedullary. One case report documents the response standard this question
wants, a PET-negative and MRD-negative remission covering marrow and an extramedullary site on venetoclax-HMA
before transplant; one patient supplies no rate.</small></td><td>44%
<small
class="persona-line">pooled response rate, not resolved to CR, for venetoclax plus HMA, mainly in secondary myeloid
sarcoma</small>
<small>pooled within a 7241-patient systematic review; subset denominators not
resolved</small>
<small class="persona-line">Pooled across 85 heterogeneous reports with mixed response
definitions. Secondary myeloid sarcoma dominates the venetoclax arm, which is her category, but nothing is stratified by
post-allograft relapse or venetoclax failure, and her own venetoclax refractoriness argues directly against
re-exposure.</small></td><td>Not extracted by the review; regimen toxicity sits with the post-allograft
HMA-venetoclax row above (30-day mortality 3.6%, grade 3-4 cytopenias 99%).</td><td><span class="fit-badge
fit-partial">Off-label</td><td><a
href="https://pubmed.ncbi.nlm.nih.gov/41463221">PMID 41463221
<a

href="https://pubmed.ncbi.nlm.nih.gov/33933047">PMID 33933047</td></tr><tr><td>13</td><td>Intermedia
cytarabine alone (MIRROS randomised control arm; context row)
<small class="persona-line">Context row, read together with the Giles second-salvage benchmark: true CR 13% (76/594, Wilson 10.3 to 15.7) across pooled second-salvage regimens, stratifying from 54% down to 0% in the highest-risk group. These two are the floor every candidate above must be read against.</small></td><td>17.1% (21/123)
<small class="persona-line">CR (true CR, TP53 wild-type intention-to-treat control arm of a randomised phase 3)</small>
<small>21/123</small>
<small>95% CI 95% CI 11.4 to 24.7 (Wilson, computed)</small>
<small class="persona-line">R/R AML after up to two prior induction regimens, not selected for post-allograft relapse or venetoclax failure. Carried as the largest randomised true-CR benchmark in R/R AML, not as a proposal; the idasanutlin arm did not beat it.</small></td><td>Control-arm profile: grade 3+ febrile neutropenia 49.3%, 30-day mortality 5.5%.</td><td>Available</td><td><a
href="https://pubmed.ncbi.nlm.nih.gov/35413116">PMID 35413116
<a
href="https://pubmed.ncbi.nlm.nih.gov/15973664">PMID 15973664</td></tr><tr><td>14</td><td>Lintuzuma
plus CLAG-M
<small class="persona-line">Not obtainable: the phase 1 completed and the follow-on venetoclax combination is suspended (both re-verified against the registry, 2026-08). The expanded-access call to the sponsor has never been made; one board member said outright that its ranking changes on a yes. This row is why the deliverable column exists: the best-evidenced candidate has no route, and that is a finding, not a recommendation.</small></td><td>56.6%
<small class="persona-line">CRc (composite CR/CRi; no CR-only split published. Subgroups, also composite: TP53-mutated 50%, prior venetoclax 38.5%, on single-digit denominators)</small>
<small>n=26 (phase 1, evaluable patients)</small>
<small class="persona-line">The best matched-subgroup signal in the file, sitting on two of her three hardest axes (TP53-aberrant, prior venetoclax), with prior transplant explicitly allowed by the protocol. Every figure is composite and the subgroup denominators are single digits.</small></td><td>What CLAG-M plus an alpha emitter would be expected to produce: grade 3-4 febrile neutropenia 65.4%, white-cell decrease 50%.</td><td>Not deliverable</td><td><a
href="https://pubmed.ncbi.nlm.nih.gov/39955432">PMID 39955432
<a
href="https://clinicaltrials.gov/study/NCT03441048">NCT03441048
<a
href="https://clinicaltrials.gov/study/NCT06802523">NCT06802523</td></tr><tr><td>15</td><td>Flotetuzumab (CD123 x CD3 bispecific DART)
<small class="persona-line">The strongest TP53-specific remission signal in the dossier and the most stranded: MacroGenics stopped development and NCT02152956 is terminated as a business decision (re-verified against the registry, 2026-08). It cannot be the answer however her TP53 workup resolves, and its headline number would not meet this question's endpoint even if it could.</small></td><td>47% (7/15)
<small class="persona-line">response defined as under 5% marrow blasts without required count recovery, in a post-hoc TP53-abnormal subgroup; not ELN CR. The parent trial's figure at the recommended dose is CR/CRh 26.7% in primary induction failure or early relapse</small>
<small>7/15 (subgroup); 30 patients at the recommended dose for the parent figure</small>
<small class="persona-line">TP53-abnormal R/R AML touches one of her pending axes, but enrolment was primary induction failure and early relapse, not late post-allograft relapse, and the subgroup is 15 patients read post hoc.</small></td><td>Infusion-related reactions and cytokine release syndrome in nearly all patients on the parent trial, largely grade 1-2, managed with stepwise dosing, dexamethasone pretreatment and prompt tocilizumab.</td><td>Not deliverable</td><td>PMID 33057635
<a
href="https://pubmed.ncbi.nlm.nih.gov/32929488">PMID 32929488
<a
href="https://clinicaltrials.gov/study/NCT02152956">NCT02152956</td></tr><tr><td>16</td><td>Uproleselan (GMI-1271) plus MEC
<small class="persona-line">One of the few true-CR splits in the file with a prior-transplant fraction attached, and no route to it: the confirmatory phase 3 (NCT03616470) was terminated after missing its primary endpoint (re-verified against the registry, 2026-08), the phase 2 signal never converted, and there is no supply.</small></td><td>35% (19/54)
<small class="persona-line">CR (true CR at the recommended phase 2 dose, reported apart from the composite; CR/CRi 41%)</small>
<small>19/54 at the recommended phase 2 dose</small>
<small class="persona-line">R/R cohort in which 18% had a prior transplant, with no venetoclax-failure stratum reported; the confirmatory phase 3 excluded secondary refractory AML, the category she now occupies after failing reinduction.</small></td><td>Moderate for an intensive backbone: grade 3+ febrile neutropenia 59%, sepsis 12%, mucositis 2% (the regimen's selling point against MEC alone), 30-day mortality 2%, 60-day 9%.</td><td>Not deliverable</td><td><a

href="https://pubmed.ncbi.nlm.nih.gov/34543383">PMID 34543383
<a
href="https://clinicaltrials.gov/study/NCT02306291">NCT02306291
<a
href="https://clinicaltrials.gov/study/NCT03616470">NCT03616470</td></tr><tr><td>17</td><td>Iomab-B
(131I-apamistamab) radioimmunoconditioning into allogeneic HCT (SIERRA)
<small
class="persona-line">Randomised proof that antigen-targeted conditioning can carry active leukaemia into a graft and a
durable CR where conventional salvage produced none; mechanism support for the rank-2 route rather than an option
for her. The successor protocol (NCT07157514) carries the prior-HCT exclusion forward and has not opened (re-verified
against the registry, 2026-08).</small></td><td>17.1% versus 0%
<small
class="persona-line">durable CR lasting at least 180 days (randomised, intention to treat), a stricter bar than every
single-timepoint CR in this table; it reads lower than it plays</small>
<small>76 versus 77
randomised</small>
<small>95% CI 95% CI 9.4 to 27.5 versus 0.0 to 4.7</small>
<small
class="persona-line">Active R/R AML at 55 and over, close to her disease state on burden and refractoriness,
except that SIERRA excluded prior allogeneic or autologous HCT, so she could not have
enrolled.</small></td><td>No worse than conventional care within the trial: grade 3+ treatment-related events
59.7% versus 59.2%, 100-day non-relapse mortality 12.2% versus 14.3%.</td><td>Not
deliverable</td><td>PMID 39298738
<a
href="https://clinicaltrials.gov/study/NCT02665065">NCT02665065
<a
href="https://clinicaltrials.gov/study/NCT07157514">NCT07157514</td></tr><tr><td>18</td><td>Pivekimab
sunirine (IMGN632), CD123-directed antibody-drug conjugate
<small class="persona-line">A confidence
interval running from 6 to 36 covers everything from negligible to moderate, too wide and too low to compete on this
question's endpoint even before the route closed: NCT03386513 is active but no longer recruiting (re-verified
against the registry, 2026-08) and there is no marketed supply.</small></td><td>17%
(5/29)
<small class="persona-line">composite CR (CRc) at the recommended phase 2 dose; no CR-only
split. ORR 21%</small>
<small>5/29</small>
<small>95% CI 95% CI 6 to 36</small>
<small
class="persona-line">CD123-positive R/R AML with ECOG 0-1 required, which she meets, and no antigen-density
threshold, which suits her qualitative-only flow; no post-allograft or venetoclax-failure subset is
reported.</small></td><td>Light at the recommended dose: grade 3+ febrile neutropenia 10%, infusion reactions
7%, anaemia 7%; two reversible dose-limiting veno-occlusive disease events in escalation, which matters ahead of a
possible second graft.</td><td>Not deliverable</td><td><a
href="https://pubmed.ncbi.nlm.nih.gov/38423051">PMID 38423051
<a
href="https://clinicaltrials.gov/study/NCT03386513">NCT03386513</td></tr><tr><td>19</td><td>Idasanutlin
plus cytarabine (MIRROS experimental arm)
<small class="persona-line">Assessed as a therapy and kept
in this file mainly for its control arm, which sits at rank 13 as the randomised benchmark. An MDM2 antagonist needs
TP53 wild-type disease, and idasanutlin's AML development stopped after MIRROS missed its primary endpoint;
NCT02545283 reads terminated (re-verified against the registry, 2026-08).</small></td><td>20.3%
(47/232)
<small class="persona-line">CR (true CR, TP53 wild-type intention to treat, randomised); it did not
beat cytarabine alone (17.1%; odds ratio 1.23, 95% CI 0.70 to 2.18), and overall survival did not move (HR
1.08)</small>
<small>47/232</small>
<small class="persona-line">R/R AML after up to two prior
inductions, not selected for post-allograft relapse or venetoclax failure; the efficacy population is TP53 wild-type by
design, which is exactly the axis her pending TP53 workup leaves unresolved.</small></td><td>Heavier than its
own control arm: grade 3+ diarrhoea 16.9% versus 6.2%, sepsis 11.3% versus 4.8% (fatal sepsis 5.3% versus 1.4%),
30-day mortality 7.9% versus 5.5%.</td><td>Not deliverable</td><td><a
href="https://pubmed.ncbi.nlm.nih.gov/35413116">PMID 35413116
<a
href="https://clinicaltrials.gov/study/NCT02545283">NCT02545283</td></tr><tr><td>20</td><td>Tagraxofusp
plus cladribine and cytarabine (NCT06561152)
<small class="persona-line">Recruiting at a single US
centre and closed to her at the door (re-verified against the registry, 2026-08). Logged so the record shows the CD123
combinations were checked rather than assumed open.</small></td><td>No results posted
<small
class="persona-line">CR and composite CR (CR/CRi/CRh) are named secondary endpoints, separated the way this
question requires; nothing has reported</small>
<small class="persona-line">CD123 positivity needs no
minimum threshold, which suits her qualitative-only flow. Enrolment admits only patients whose sole prior therapy was
venetoclax plus a hypomethylating agent, so FLAG-IDA plus venetoclax and a prior allograft exclude her
outright.</small></td><td>Unpublished for this combination; tagraxofusp's label carries a boxed warning for
capillary leak syndrome.</td><td>Not deliverable</td><td><a

href="https://clinicaltrials.gov/study/NCT06561152">NCT06561152</td></tr><tr><td>21</td><td>Venetoclax plus CLAG-M (NCT06660368; NCT04797767)
<small class="persona-line">NCT04797767 names extramedullary myeloid sarcoma as a qualifying disease manifestation for relapsed patients, which almost no other protocol does, and that door is the one that has closed. Her venetoclax refractoriness also argues against a venetoclax-containing reinduction on any protocol. Both statuses re-verified against the registry, 2026-08.</small></td><td>No results posted on either protocol
<small class="persona-line">MRD-negative remission is the primary endpoint of the recruiting R/R protocol; no CR figure exists for the combination</small>
<small class="persona-line">The recruiting R/R protocol trips four of her features: venetoclax-refractory disease, prior high-dose cytarabine with FLAG named, any known TP53 mutation, and a WBC ceiling of 25,000; extramedullary AML with marrow involvement is the one axis she clears. The other protocol admitted R/R patients to its phase 1 only, and the phase 2 that continues to recruit takes newly diagnosed adverse-risk disease.</small></td><td>Unpublished; expect at least the CLAG-M backbone rates (severe infection 45%, early death 7%) plus venetoclax-driven cytopenias.</td><td>Not deliverable</td><td>NCT06660368
NCT04797767</td></tr><tr><td>22</td><td>Venetoclax added to FLAMSA-treosulfan sequential conditioning (NCT05807932)
<small class="persona-line">The venetoclax-era descendant of the FLAMSA figure behind rank 2, logged so the record shows the modern sequential-conditioning platform was checked rather than assumed open to her. Recruiting at a single German centre (re-verified against the registry, 2026-08).</small></td><td>No results posted
<small class="persona-line">the primary endpoint is 30-day organ toxicity, not remission; even a matched patient would get no CR rate from this protocol</small>
<small class="persona-line">Enrolment requires disease untreated beyond hydroxyurea or at most two hypomethylating-agent courses with or without venetoclax; FLAG-IDA plus venetoclax and her prior allograft exclude her at the front door.</small></td><td>Unpublished; safety within 30 days of transplant is the question the trial itself is asking.</td><td>Not deliverable</td><td>NCT05807932</td></tr><tr><td>23</td><td>VAM (venetoclax-containing combination) in extramedullary and secondary AML (NCT07028086)
<small class="persona-line">Its existence answers a question this table keeps running into: is anyone measuring remission in myeloid sarcoma prospectively? One recruiting single-centre front-line study, and she cannot enter it (re-verified against the registry, 2026-08).</small></td><td>No results posted
<small class="persona-line">composite CR is the primary endpoint; nothing has reported</small>
<small class="persona-line">The only protocol found in this search that treats extramedullary disease and myeloid sarcoma as an enrolment category rather than an afterthought, and it is restricted to previously untreated AML with an age ceiling of 65, at a single centre in Shanghai.</small></td><td>Unpublished; the registry record does not specify dosing.</td><td>Not deliverable</td><td>NCT07028086</td></tr><tr><td>24</td><td>IMPACT-AML randomised platform, high- against low-intensity physician's-choice salvage (NCT06713837)
<small class="persona-line">Last because it adds no distinct therapy: its arms are regimens already in this table. It is the one randomised comparison recruiting on exactly the discrimination gap this verdict names, so if it reports, criterion 7 gains the direct evidence this file lacks. Recruiting (re-verified against the registry, 2026-08).</small></td><td>No results posted
<small class="persona-line">event-free survival is the primary endpoint; remission rates are collected as secondary</small>
<small>339 planned</small>
<small class="persona-line">First or second relapse or refractory AML, which contains her line; entry requires the treating physician to consider both arms reasonable, a judgement hard to defend at 85% marrow blasts after an allograft, and the sites are Italian.</small></td><td>Not applicable at the platform level; each arm carries the toxicity of the physician's-choice regimen given, most of which appear in the rows above.</td><td>Trial only</td><td>NCT06713837</td></tr></tbody></table>

What would have answered this, decided before the search

<table class="trial-table"><thead><tr><th>Pre-registered criterion</th><th>Result</th><th>Points toward</th><th>What was found</th></tr></thead><tbody><tr><td>A prospective trial or multicentre cohort (n of 20 or more) reports a true CR rate (ELN CR, not composite) for a deliverable candidate in a population that includes post-allo-HCT relapse or prior

intensive-chemotherapy-plus-venetoclax failure, and that rate is clearly higher than the alternatives' rates in comparably matched populations.</td><td>Not met</td><td>yes</td><td>Not found. The one prospective true-CR figure from a population that includes her hard axes is CPX-351 plus venetoclax at 15% (5/33), which sits below the unmatched competitors rather than above them, and its Wilson interval (6.7 to 30.9) contains the second-salvage floor and every alternative. No deliverable candidate reports a matched true CR that clears the field.</td></tr><tr><td>A transplant-integrated platform (sequential FLAMSA-type conditioning into a second allograft, or targeted radioimmunotherapy conditioning) reports remission rates in active-disease relapse, applicable after a prior allograft, that meet or beat the best standalone reinduction. If met, the answer moves away from any standalone regimen toward proceeding directly to conditioning, which also renders 'induce CR first' partly moot.</td><td>Not met</td><td>depends</td><td>Not met as pre-registered, which required applicability after a prior allograft. FLAMSA's 88% true CR (66/75) is overwhelmingly first transplants and predates venetoclax-era salvage; ASAP excluded prior allo-HCT, randomised patients heading to a first graft, and did not show non-inferiority (p=0.036); SIERRA excluded prior HCT outright; and the two open platforms that would take her (NCT06928662, NCT04375631) have published no remission data and read out mortality and disease-free survival rather than CR. The platform's supporting logic survives (see the no-CR-precondition finding carried in the answer), but no post-allograft remission number exists.</td></tr><tr><td>The leading candidate's CR figures come from populations that exclude her on a load-bearing axis (post-allo relapse excluded, blast cap below her burden, venetoclax-naive, first-relapse rather than refractory, or TP53-wild-type-enriched) with no matched data to fall back on.</td><td>Met</td><td>no</td><td>Lands for CLAG-M and CLAG-GO, the backbone the board most often ranked first: the 58% true CR is pre-venetoclax with no post-allograft subgroup and no matched series to fall back on, and CLAG-GO itself has zero published results in refractory AML (its only efficacy paper is frontline core-binding-factor disease). It does not land for CPX-351 plus venetoclax, whose cohort included her kind of patient, which is exactly why its 15% is the most transferable figure and one of the lowest.</td></tr><tr><td>Across the deliverable candidates, CR rates in matched populations are indistinguishable: overlapping confidence intervals, no comparative data, no candidate separating from the rest. If met, the honest verdict is that the evidence does not discriminate and the question has no single winner.</td><td>Met</td><td>no</td><td>Lands, and it is the central determinant of this answer. Wilson intervals on true CR in every population containing her overlap: CPX-351 plus venetoclax 6.7 to 30.9 (5/33), post-allograft venetoclax salvage 11.8 to 48.8 (5/19), the MIRROS randomised control arm 11.4 to 24.7 (21/123), and the Giles second-salvage benchmark 10.3 to 15.7 (76/594). Only composite endpoints (RELAX CRc 61.7 to 84.2) or unmatched populations (CLAG-M 48.7 to 66.6, on the corrected 66/114 evaluable) clear the pack, and both separations are artefacts of the comparison. On demonstrated CR probability the question does not discriminate among deliverable candidates, so this run's answer shape was downgraded from a ranked list to a bare verdict. Each persona's round-1 list in fact ordered by a different axis: magnitude (risktaker), deliverability plus documented toxicity (conservative), eligibility (advocate), evidence transferability (critic), indication fit (concensusite). None ordered by demonstrated CR probability, because nothing published permits it.</td></tr><tr><td>At least one source for a leading candidate assessed extramedullary disease at response, or reports responses in myeloid sarcoma. If met, a full-remission claim covering marrow and thigh is supportable; if unmet, every CR claim in the answer carries a marrow-only caveat.</td><td>Not met</td><td>either</td><td>Unmet in every way that matters, and re-scored downward during round 2. The only source that measured a CD33-positive extramedullary lesion at response is two paediatric patients on gemtuzumab followed by serial FDG-PET/CT, with neither response graded as CR; the largest adult myeloid sarcoma series (45% CR in 43 intensively treated patients) does not state whether extramedullary sites were assessed. The round-2 re-score cuts deeper than the pre-registered caveat: ELN 2022 requires absence of extramedullary disease within the CR definition itself, so every marrow-only CR figure in this dossier fails the question's own definition for her rather than merely needing a footnote. The serial FDG-PET/CT method is worth copying even though the two-patient result is not a rate.</td></tr><tr><td>The agent with the best matched-population CR signal is not deliverable: discontinued, no open site, or excluded by her post-allo status, blast burden, ECOG or undocumented organ function. If met, the verdict names the best deliverable option instead and says what was passed over and why.</td><td>Met</td><td>depends</td><td>Lands. Lintuzumab-Ac225 plus CLAG-M holds the best matched-subgroup signal in the file (composite CR/CRi 56.6% overall, 50% TP53-mutated, 38.5% prior-venetoclax, single-digit subgroup denominators) and the best mechanism fit (alpha-particle kill independent of cell-cycle position and p53, with in-vivo reversal of venetoclax resistance), and it is not deliverable: NCT03441048 completed and the follow-on NCT06802523 is suspended, both re-verified against

ClinicalTrials.gov on 2026-08. Because criterion 4 also landed, no single best deliverable option exists to name in its place; the deliverable convergence is the trial route, led by CLAG-GO on NCT04050280 on eligibility rather than evidence. Carried as an open action rather than a footnote: the expanded-access call to the sponsor has never been made, two personas pressed for it, and one said its ranking changes if the answer is yes. Flotetuzumab (47% response in TP53-abnormal disease, 7/15, post-hoc, non-ELN definition) and uproleselan (true CR 35% with a failed confirmatory phase 3) were passed over for the same deliverability reason.</td></tr><tr><td>Direct comparative evidence, randomised or matched-cohort, on remission induction between two or more deliverable candidates in relapsed/refractory AML. If met, the ranking can rest on a comparison rather than on cross-trial arithmetic.</td><td>Met</td><td>either</td><td>Met only by Price 2011: CLAG 37.9% versus MEC 23.8% true CR (p=0.048, n=162), a single-centre retrospective chart review with physician-assigned treatment and no adjustment. It is the sole head-to-head in the file and too weak to rank on; it leans the same direction as the blast-burden gradient against MEC at her disease load, and no stronger comparison exists among deliverable candidates.</td></tr></tbody></table>

Evidence both ways

<table class="trial-table"><thead><tr><th>Direction</th><th>Finding</th><th>Strength</th><th>Population match</th></tr></thead><tbody><tr><td>For</td><td>On demonstrated true CR in every population that contains her, no deliverable candidate separates from the rest. The Wilson intervals all overlap: CPX-351 plus venetoclax 6.7 to 30.9 (5/33 true CR, in a cohort with 58% prior venetoclax, 30% prior transplant, 44% at second salvage or later), post-allograft venetoclax salvage 11.8 to 48.8 (5/19), the randomised MIRROS control arm of cytarabine alone 11.4 to 24.7 (21/123), and the second-salvage benchmark 10.3 to 15.7 (76/594). The intervals that do clear the pack belong to composite endpoints or to populations she is not in, so both separations are artefacts of the comparison rather than findings about her.</td><td>strong</td><td>These are the only true-CR figures in the file from populations that include post-allograft relapse, prior venetoclax failure, or second salvage, which are her load-bearing axes. The non-discrimination claim rests on matched evidence; the cross-trial arithmetic is the weakness and it was named, not laundered.</td></tr><tr><td>For</td><td>ELN 2022 puts absence of extramedullary disease inside the CR definition; it omits only the assessment modality and timing. Every CR figure in this dossier was assessed on marrow alone, so none of them meets the question's endpoint for a patient with a documented thigh myeloid sarcoma. The supporting biology cuts the same way: 56% of paired myeloid-sarcoma and marrow samples carry discordant mutation profiles, and 46% of post-transplant relapses in the pooled literature were isolated extramedullary, so a marrow answer cannot be assumed to cover the thigh.</td><td>strong</td><td>The definitional point applies to her exactly; the discordance and isolated-relapse figures come from pooled myeloid sarcoma cohorts, adjacent rather than identical to her situation.</td></tr><tr><td>For</td><td>The guideline position converges with the evidence position. No guideline endorses any regimen for second salvage after intensive chemotherapy plus venetoclax in a post-allograft patient; NCCN has routed relapsed/refractory AML to a clinical trial or R/R salvage since at least v3.2019 and the v3.2026 Insights change nothing for her line; Vyxeos is labelled for newly diagnosed t-AML/AML-MRC only, which makes the board's one dissenting rank-1 off-indication in second salvage. Guideline silence and overlapping intervals arrive at the same place: this patient belongs on a protocol.</td><td>moderate</td><td>Guideline statements about her exact line, checked live against current versions during the board rounds.</td></tr><tr><td>For</td><td>The second-salvage benchmark bounds what any deliverable regimen has demonstrated: 13% true CR across 594 patients at second salvage, stratifying from 54% down to 0% in the highest-risk group. It bounds rather than estimates her odds in both directions, because her six-year first remission is a favourable factor on that model while her blast burden and non-inv(16) disease count against.</td><td>moderate</td><td>Second salvage is her line; the cohort is single-centre, 1980 to 2004, and contains no venetoclax-era or post-allograft-specific stratum.</td></tr><tr><td>Against</td><td>HAM plus venetoclax (RELAX) reported composite CR/CRi of 75% (41/55) with the largest post-transplant fraction of any prospective salvage cohort here (19/55), and its interval (61.7 to 84.2) does clear the matched pack. If the unpublished CR-only split held up in the post-allograft subgroup, a candidate would separate and the verdict would move. Against relying on it now: the composite is the only figure published, the registry entry posts no results, the abstract's serious-adverse-event line does not reconcile (15 SAEs in 14 patients printed as 13%, where 14/55 is 25%), her second-salvage venetoclax-refractory state fell outside enrolment, and the trial's recruitment status reads unknown.</td><td>moderate</td><td>Closest prospective cohort on the transplant axis, but enrolment required relapse from CR or failure of standard induction, not refractoriness to intensive chemotherapy plus venetoclax at second

salvage.</td></tr><tr><td>Against</td><td>CLAG-M carries the largest true-CR figure on a deliverable backbone, 58% (66 of 114 evaluable; the often-printed 66/118 gives 55.9%), and the only head-to-head in the file favours a cladribine backbone over MEC on true CR (37.9% versus 23.8%, p=0.048). If that read-across transferred to her, a winner would exist.</td><td>moderate</td><td>It does not transfer cleanly: the 2008 cohort closed before venetoclax existed, reports no post-allograft subgroup, scored CR by pre-ELN morphologic criteria, and WBC above 10 g/L predicted failure where her peripheral blasts ran 58 to 82%. The head-to-head is a single-centre unadjusted retrospective with physician-assigned treatment.</td></tr><tr><td>Against</td><td>Remission can be manufactured by conditioning rather than bought first: FLAMSA sequential conditioning produced 88% true CR (66/75) in refractory disease, and ASAP's randomised comparison found day-56 post-transplant CR of 83% without induction against 79% with it. If those numbers applied after a prior allograft, the transplant-integrated platform would beat every standalone reinduction and partly moot the question.</td><td>moderate</td><td>Neither applies to her as published: FLAMSA was overwhelmingly first transplants two decades before venetoclax-era salvage, ASAP excluded prior allo-HCT and did not show non-inferiority (p=0.036), and both open US platforms that would take her (NCT06928662, NCT04375631) read out mortality and disease-free survival, not CR.</td></tr><tr><td>Against</td><td>Two candidates carry matched-subgroup signals that could have separated. Lintuzumab-Ac225 plus CLAG-M reported composite CR/CRi of 56.6% overall, 50% in TP53-mutated and 38.5% in prior-venetoclax patients, with in-vivo reversal of venetoclax resistance behind it, but every figure is composite on single-digit subgroup denominators and the agent is not deliverable. HMA plus venetoclax in post-allograft relapse reported 48% composite CR/CRi (40/83) with 3.6% 30-day mortality, and it is the only series where prior venetoclax failure was tested against outcome and did not predict it, but no CR-only split exists and the median relapse in that cohort came at 7 months post-transplant with low disease burden, against her six years and roughly 85% blasts.</td><td>weak</td><td>The lintuzumab subgroups sit on two of her three hardest axes but are single-digit; the HMA-venetoclax cohort matches her transplant axis and mismatches her burden, kinetics and refractoriness.</td></tr></tbody></table>

What would change this answer

- The expanded-access call. If the sponsor of lintuzumab-Ac225 (NCT03441048 completed; NCT06802523 suspended) will supply the agent, the best matched-subgroup signal in the file becomes deliverable and this verdict gets revisited; the board member who ranked by magnitude said outright that its ranking changes on a yes. Nobody has made the call.
- A CR-only split from RELAX (HAM plus venetoclax), ideally by post-transplant subgroup, together with a reconciled serious-adverse-event denominator. Its composite interval is the one deliverable figure that clears the matched pack; a true-CR figure that held up would separate a candidate.
- Published remission results from NCT04050280 (CLAG-GO) in refractory AML. The protocol the board converged on currently has no number of its own; its first readout replaces a borrowed backbone figure with an owned one.
- The gating workup landing: organ-function labs (creatinine clearance, LFTs, echocardiogram), quantitative CD33/CD123 density, and TP53 variant resolution on a quantitative assay with CNV coverage. Each closes or opens specific doors, and the TP53 burden split (composite CR/CRi 41% below 20% VAF versus 13% at or above) would move the expected rate for whatever is given.
- Thigh imaging built into response assessment, MRI or serial FDG-PET/CT, for whichever therapy is chosen. Without it no resulting remission can be graded against ELN 2022 for her, and with it the run's central definitional objection is answerable in her own case.
- Her answer to two questions nobody has asked: whether she would accept roughly a 13 to 17% chance of the intended result against 7 to 21% sixty-day mortality, and whether she would go straight to sequential conditioning without a CR first. Either answer reorders what 'best' means here, because once demonstrated benefit stops discriminating, the choice runs on cost and goal.
- Any prospective remission data in active-disease relapse after a prior allograft from a sequential-conditioning platform. Both open protocols read out mortality and disease-free survival, so this data will have to come from elsewhere; if it met or beat standalone reinduction, criterion 2 would land retroactively and the platform would become the answer.

Where the board disagreed

<table class="trial-table"><thead><tr><th>Board seat</th><th>Position</th><th>Into the answer</th></tr></thead><tbody><tr><td>critic</td><td>Ranked CPX-351 plus venetoclax first as the only prospective cohort that enrolled her kind of patient and still published a true CR apart from the composite, then computed the criterion-4 interval analysis that undoes every ranking including its own, and pressed it in all four round-2 rows: no persona's list orders by demonstrated CR probability, because nothing published permits it. Conceded in round 2 that the Vyxeos label (newly diagnosed t-AML/AML-MRC only) makes the pick off-indication in second salvage, stripping the off-protocol fallback and leaving it viable only as a trial slot. The rank-1 was not carried; the interval analysis is the verdict.</td><td>carried</td></tr><tr><td>risktaker</td><td>Ranked sequential conditioning into a second allograft first, arguing an 88% with a flawed denominator beats a well-matched 13% that predicts failure, and conceded criterion 2's finding while defending the platform rather than the number. Two of its points carried: the Giles asymmetry (if 58% is not her number for lack of her hard features, 13% is not hers either, since Giles credits her six-year first remission) now qualifies how the benchmark is stated, and the lintuzumab-Ac225 expanded-access call is carried as an open action. The rank-1 itself was not carried as the answer because no post-allograft remission data exists for the platform, but the route survives in the answer as the live alternative no guideline forecloses.</td><td>carried</td></tr><tr><td>conservative</td><td>Pre-declared a threshold veto on any intensive rank-1 delivered without documented organ function, then withdrew it on the record in round 2 for on-protocol options because trial entry screens supply the labs before the drugs. Converged with advocate on the finding that once matched benefit stops discriminating, cost separates the options, favouring the protocol route with no anthracycline and the lowest documented early mortality. Carried: the answer conditions every intensive option on the organ-function screen and names cost as the discriminating axis.</td><td>carried</td></tr><tr><td>advocate</td><td>Argued that criterion 4 indicts every ordering including its own: no list under a heading promising 'most likely to induce a complete remission' is honestly ordered by that promise, which is the argument that drove the downgrade to a bare verdict. Named the consent gap (an empty toxicity_vetoes array records that nobody asked, not that she declined) and the unmeasured axis (nothing in the scope spine counts what treatment costs her in weeks of life; ASAP's 27 hospital days appears as evidence, never as an endpoint). Both carried into the answer and into what would change it.</td><td>carried</td></tr><tr><td>concensusite</td><td>Surfaced the two findings that re-scored the run: ELN 2022 puts absence of extramedullary disease inside the CR definition, which fails every marrow-only figure in the file against the question's own endpoint, and no guideline writes a CR precondition into second transplant, which is the one place the board questioned the goal rather than the options. Made the convergence explicit: guideline silence and overlapping intervals arrive at the same place, a protocol slot. All three carried; the rank-2 gemtuzumab monotherapy was not, after two personas objected that on-label supply against 85% marrow blasts is the on-label way to undertreat her.</td><td>carried</td></tr></tbody></table>

!!! warning "What this run did not assess" This run assessed one thing: the published evidence for inducing a complete remission with her next line of therapy. It did not assess durability of remission, relapse-free or overall survival, choice of consolidation, second-transplant platform, DLI timing, or the HLA-restricted cellular options; those belong to the source case (aml-mds-related-rr-tp53-aberrant-hla-pending-x7q2-rerun2), whose ranking was not revisited here and remains as published, including its rank-1 gating workup, which proceeds concurrently with whatever is chosen. Sequencing consequences, whether a candidate forecloses later options, were weighed in the source ranking and not re-litigated. Supportive and bridging measures without remission intent were out of scope. 'No single best-evidenced option' is a statement about the evidence, not about her odds on any given regimen, and it must not be read as 'nothing worth giving': every deliverable candidate retains a real, if undifferentiated, chance of remission, and the trial route the board converged on is a positive recommendation, not a shrug. A reader who wants 'best next move' rather than 'most likely to produce a CR' needs the source dossier open beside this answer. The candidates table carries every therapy the research tier assessed, including those with no route to her; it excludes only non-therapy context rows, and says which: the Giles pooled second-salvage benchmark (quoted as the floor in the rank-13 row's notes), the TP53-aberrant and TP53-frontline salvage benchmarks, the first-relapse prognostic index, and the pre-transplant remission-depth series, all of which describe her odds rather than anything she could receive. ASAP, the randomised watchful-waiting comparison, is a strategy rather than a therapy and its figures live inside the rank-2 row rather than in a row of their own.

- The source case's ranking. Its endpoint was overall curative-intent benefit and it stands as published; nothing here re-opens, re-ranks or amends it. A therapy ranked high there may rank low here and the reverse, because CR likelihood and curative benefit are different questions.
- Durability of remission, relapse-free survival, overall survival, and everything after CR: choice of consolidation,

second-transplant platform, DLI timing, and the HLA-restricted cellular products. Those were the source case's territory. The one carve-out is that where a transplant-integrated platform induces remission as part of the procedure, its remission data are in scope; its survival data are not this question's endpoint.
- Whether a candidate forecloses later options. Sequencing consequences were weighed in the source ranking and are not re-litigated here; a reader who wants them should read the source dossier alongside this answer.
- The gating workup itself (HLA typing, HA-1/HA-2 genotyping, TP53 variant resolution, antigen density, organ-function labs). It stays rank 1 of the source case, assumed to proceed concurrently; it is not a therapy and is not re-adjudicated.
- Supportive and bridging measures that control counts without remission intent, such as hydroxyurea or leukapheresis.
- Best therapy overall. The answer here is 'most likely to produce a CR', and a reader must not convert that into 'best next move' without the durability and sequencing analysis this run deliberately excludes.

Downloads

- Question report (offline HTML) — the same answer, self-contained HTML that opens offline
- Question report (PDF) — print-friendly